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Bristol-Myers Squibb Company Presents at 44th Annual J.P. Morgan Healthcare Conference, Jan-12-2026 07:30 AM

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Transcript Summary

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Theme 1: Financial Discipline and Capital Allocation

The company emphasized the importance of being financially disciplined in its operations and capital allocation. They are focused on business development as a top priority to improve the company's growth profile. The goal is to invest wisely in opportunities that align with their scientific capabilities and can add value. Specific mention was made of a \$2 billion cost optimization initiative that is well underway, with \$1.5 billion already achieved.

Theme 2: Growth Portfolio Performance

The growth portfolio increased by 17% in the first nine months of the previous year. This portfolio is designed to consist of multiple differentiated products that are early in their life cycle, indicating a long runway for growth. Notably, four products are annualizing sales of over \$1 billion each, including Opdualag, Breyanzi, Camzyos, and Reblozyl, with Reblozyl exceeding \$2 billion in sales.

Theme 3: R&D Execution and Pipeline Development

The company is committed to driving execution across research and development (R&D) to ensure that clinical programs are delivered on time and with high success rates. They are focused on areas where they can add value and have made significant changes to their R&D organization to improve outcomes. The early-stage pipeline includes promising assets like CD19 NEX-T for autoimmune diseases and CELMoDs for prostate cancer and lymphoma.

Theme 4: Strategic Business Development

The company completed five strategic business development deals last year, aimed at enhancing long-term growth. These deals are part of a broader strategy to ensure that the company can stand in front of investors and demonstrate the added value these assets bring. They are prioritizing opportunities in phases I, II, and III of development for near-term growth.

Theme 5: Organizational Changes and Efficiency

Significant changes have been made within the organization to enhance operational efficiency and focus on execution. The company has adopted a mantra called the "say-to-do ratio," ensuring that they deliver on

commitments made to investors. Efforts are underway to rewire operations and leverage technology, including AI, to optimize performance and outcomes.

Theme 6: Future Growth and Confidence

Looking ahead to 2026, the company remains confident in its ability to execute the growth plan. They highlighted the importance of maintaining momentum in the growth portfolio and ensuring that all investments have clear scientific rationale and commercial value. The leadership expressed optimism about their progress and strategic flexibility, positioning the company for sustainable growth into the next decade. ""

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Original Document

Call Participants

Executives

Christopher Boerner

Analysts

Christopher Schott

Presentation

Christopher Schott

Good morning, everybody. I'm Chris Schott from JPMorgan and welcome to the 44th Annual JPMorgan Healthcare Conference. It's great to see everybody today. So it's my pleasure to be again -- to be introducing Bristol-Myers to again kick off the conference this year. From the company, we have Chairman and CEO, Chris Boerner. Chris is going to do a quick presentation and then we're going to go to a fireside format from there. So with that, we'll turn it over to Chris. Happy New Year and looking forward to the presentation. Thanks, Chris.

Christopher Boerner

Thank you, Chris, for the introduction, and it is fantastic to be back kicking off another JPMorgan Healthcare Conference. Obviously, we're going to be making some forward-looking statements today, so these are my disclosures. Let me start with just a reminder of our overarching goal as a company. We remain focused on building a company that's strong financially and has the substrate to deliver industry-leading sustainable growth heading into the 2030s and beyond. Before discussing 2026 and the road ahead, let me start by quickly highlighting the progress we made against that goal last year. 2025 was a year that was focused on execution. And we have made meaningful progress across a number of different fronts. Our growth portfolio increased 17% in the first 9 months of the year. And remember, this is a portfolio that is designed to be comprised of multiple differentiated products that are earlier in their life cycle, so they have a long runway ahead of them. And as a result, our future growth will not be dependent upon 1 or 2 blockbuster products. In fact, we now have 4 products from that portfolio that are annualizing with sales of over \$1 billion each and that includes Opdualag in melanoma, Breyanzi, our CAR-T therapy in lymphoma, Camzyos, the first-in-class treatment for obstructive HCM and Reblozyl, which is actually annualizing to over \$2 billion in sales. And each of these products continues to be a core element of how we're going to grow the company and each has good momentum coming into this year. Beyond commercial performance, we made important pipeline progress last year and I'm going to speak to a subset of that pipeline in a moment. We augmented that pipeline last year with strategic business development, completing 5 deals, all designed to meaningfully add to the long-term growth profile of the company and we remain highly disciplined in the process. Our strategic flexibility to do business development was enabled by continued financial discipline. We are well on our way to delivering on the \$2 billion cost optimization that we announced roughly this time last year. And that progress includes a full-court press to rewire how we operate as a company and to make full use of technology, including AI across the enterprise. As a result of all of these efforts, we exit 2025 as a much stronger company. We have a strong balance sheet. We've reduced our debt obligation, thanks to achieving our \$10 billion debt pay down ahead of schedule. We have very strong cash flow because of the commercial performance we've had. And we had good momentum across the business and we anticipate that momentum carrying into this year. Speaking of this year, it's going to be a very busy year across multiple fronts, notably with respect to the breadth and depth of our pipeline. Let me highlight a few late-stage assets across all of our therapeutic areas with anticipated readouts this year that we believe has the potential to deliver significant value to patients and to the company. And let's start with neuroscience. Cobenfy was approved just over a year ago as the first new mechanism of action for the treatment of schizophrenia in decades. Beyond schizophrenia, we have a broad development program underway. In Alzheimer's disease psychosis, we expect readouts from our pivotal ADEPT-1, 2 and 4 studies later this year. And I think as many of you know, this is a devastating condition with upwards of 40% of Alzheimer's patients experiencing psychosis symptoms with really no great treatment options available today. We're also pursuing programs in Alzheimer's agitation and cognition, Bipolar I Disorder and Autism Irritability. Each of these represents a multibillion-dollar opportunity. In cardiovascular disease, milvexian is our Factor Xla inhibitor that has the potential to redefine anticoagulation therapy for thrombotic disease. In fact, milvexan is uniquely positioned to be the only Factor Xla -- oral Factor Xla for atrial fibrillation. And potentially the -- a best-in-class treatment in secondary stroke prevention where anticoagulants are not approved today. We expect to share Phase III

results for both of these indications in the second half of this year. In immunology, admilparant has the potential to redefine the standard of care in pulmonary fibrosis as a differentiated treatment for IPF and PPF. The lead program in IPF is progressing well and we expect to share Phase III results this year. And we expect PPF data to follow shortly thereafter in early 2027. In oncology, we're developing pumitamig, our PD-L1 VEGF bispecific with our partner, BioNTech. This is a space, obviously, we know very well, and we believe pumitamig can redefine the standard of care across multiple tumor types. Our strategy with this asset is to be either first or second in each of the indications that we're pursuing. By the end of this year, we expect to add 3 new registrational trials, including a broader reach across 2 non-small cell lung indications and a study in head and neck cancer. We are also moving forward with earlier-stage novel combinations, including with ADCs, for example, our ADC iza-bren in collaboration with SystImmune, with other I-O therapies such as BioNTech's mRNA cancer immunotherapy and with novel targeted therapies, including a study with our PRMT5 inhibitor. In hematology, iberdomide and mezigdomide are 2 oral CELMoDs in multiple myeloma. Based on the data we have seen so far, we are increasingly confident of the potential for both of these products to become a standard of care. Iberdomide has already shown a significant improvement in MRD negativity rates and we expect PFS data later this year. We also expect to see the first Phase III readout for mezigdomide later this year. And what we really like about both of these assets and in fact, this platform generally is the potential for these degraders to be combined with standard of care as well as with newer treatments like CAR-T and T-cell engineered antibodies. Each of the programs on this slide targets large, underserved patient populations. These products are novel and differentiated from existing and future competitor products. And as a result, we believe they all have the potential to deliver multibillion-dollar peak sales. Collectively, they represent a critical leg of the stool of how we strengthen the long-term trajectory of the company. And while we'll see data with each of these programs this year, these assets are part of a much larger catalyst pool that gives us multiple shots on goal across new molecules, life cycle management opportunities and key early-stage programs. The breadth and depth of these opportunities is illustrated on this slide. In 2026 alone, we expect registrational data for 6 potential new products, milvexian, admilparant, iberdomide, mezigdomide, arlo-cel and RYZ101, as well as meaningful pivotal line extension readouts for SOTYKTU and Cobenfy. Most of these readouts will occur in the back half of this year. Importantly, while the science needs to play out here, based on the totality of the data that we have seen so far across these programs, we are very eager to see these readouts and we'll keep you updated on the progress. And of course, keep in mind, we have more key data readouts coming beyond 2026, the totality of which portends a wave of launch opportunities that will take place between now and the end of this decade as vividly illustrated on this slide. This is a data-rich period where we have the potential for more than 10 new medicines and over 30 meaningful launch opportunities by 2030. These launches add additional growth drivers to the impressive performance that we've already seen in our existing portfolio and which I referenced earlier. And they expand our commercial presence in each of our core therapeutic areas. Taken together, what emerges is a younger and more diversified portfolio that will provide an impressive foundation for sustained growth leading into 2030 and beyond, which, as I noted earlier, remains our guiding objective. Now we have to deliver on that promise, and that means ensuring that we have an R&D organization that not only is adept at identifying promising areas of science but also consistently execute in a way that translates that science into new medicines with the highest probability

of success. To that end, Cristian Massacesi, joined us last year as our Chief Medical Officer and Head of Development. Cristian and his team are evolving our approach to development to exquisitely focus on 3 areas: great science, execution and value. On science, we're going to continue to prioritize exciting areas that we know well with a focus on building depth in each of our core therapeutic areas. With respect to execution, we've taken steps to increase the probability of success, applying more rigor to the design and execution of all of our clinical trials. We're also using AI and machine learning to power our R&D engine and bring treatments to patients faster. And finally, on value. We continue to be very disciplined in prioritizing programs where the science is highly differentiated and there is significant commercial value. You've seen that on display recently and how we've evolved our portfolio and you can expect that focus to continue. Over the last 3 years, we focused on making execution stronger across the entire organization. Ultimately, our goal here is to ensure that we're investing our resources in the best science, enabling us to bring important medicines to the market with the highest probability of success and delivering ultimately more value to patients, to BMS and to our shareholders. While we have more work to do, I'm very proud of the progress that we have made so far. We have strong financial flexibility, which drives strategic flexibility, enabling us to invest in our business and deliver value to shareholders. In fact, we have returned over \$22 billion to shareholders over the last 3 years, and we've increased our dividend 17 consecutive years. Further, we continue to bring exciting science into the company through business development. We did over the past 24 months, completing \$30 billion of deals, including 5 deals in 2025 that I referenced earlier. So in conclusion, we're doing what we said we would do. We've strengthened our execution across the entire company. We've built momentum into our growth portfolio and we expect that to continue into this year. We've prosecuted a pipeline of differentiated assets. That pipeline is now within months of a barrage of meaningful catalysts that kick off a wave of potential new product launches and approvals between now and the end of the decade. And finally, we've strengthened the company financially, injecting strategic flexibility that enable multiple paths to value creation going forward. I'm incredibly proud of the foundation we've strengthened coming into this year, which wouldn't have been possible were it not for the commitment and dedication of my colleagues at BMS. We like the setup that we have for 2026 and I very much look forward to keeping you updated on our progress. And with that, Chris, I will turn it over to you for questions. Thank you.

Christopher Schott

Great. Well, appreciate those remarks. Maybe just open up the conversation. I guess you're about 2 years into the CEO role now. So maybe just talk a little bit about in that -- in those 2 years, what's gone better than expected? What are areas where you still think the company needs to make improvement? And just kind of as you turn the page to 2026, what are you most focused on in your role?

Christopher Boerner

Well, what I would say in aggregate is that when you look across all of the puts and takes, I feel very good about where we are as a company and I'm confident on executing against the plan and ultimately achieving the objective I just talked about. In terms of things that have gone better, look, I think that I highlighted the growth portfolio. We're seeing very good progress in that portfolio through the first 9 months of last year,

double-digit 17% growth. We'll talk more about how we ended the year in a few weeks. But that portfolio is performing very well. It's got good momentum across a number of key products. And of course, that provides a nice foundation for how we think about growing this company in the back half of the decade and beyond. So I feel very good about where we are there. I highlighted just before that we're making very good progress in terms of taking costs out of the system. We're well on our way to delivering on the \$2 billion that we announced last year. We had \$1.5 billion of taking costs out that we announced in 2024. But more important than just the numbers is that we built this muscle around being very financially disciplined, making sure that we're spending our resources in the right place and whether we're dropping that -- those cost savings to the bottom line or reinvesting in the business, that financial flexibility is super important for a company like ours because it gives us strategic flexibility. So I feel great about that. And third, I also feel very good about the early-stage pipeline, which we don't talk a lot about. But if you look across some of the assets that are going to become increasingly important, particularly as these Phase IIIs play out, we've got an exciting program with CD19 NEX-T. The data there in autoimmune disease looks fantastic. We've got additional CELMoDs, whether it's our prostate cancer CELMoD AR LDD or golcadomide in lymphoma, which might very well be a sleeper because the data there looks quite impressive as well. So that early-stage program continues to progress, which I'm quite happy about. Obviously, the things that haven't gone well, we had some setbacks on the development side in 2025. But you bring in the right team, you focus and double down on execution. That's what Cristian and his team have done. I feel good about the progress we've made there. And that's part of just an overall focus that we have around execution, which I would say wraps around all of what I just described. So net-net, I feel good about where we are. And our priorities for 2026, not surprisingly, are the same, continue to drive commercial execution, continue to prosecute that pipeline, maintain financial discipline that gives us the strategic flexibility to continue to bring great science into the company. If we execute, I feel good about where we are.

Christopher Schott

Great. Maybe just building on that, I think in this discussion last year, I asked you how confident you were about Bristol's ability to manage through the patent expirations that are coming up. I guess just how has that view changed as you've gone through 2025? You mentioned some products moved forward and there were some setbacks. But just your level of confidence right now and as we think about those next 5 or 6 years of LOEs.

Christopher Boerner

Well, my confidence is still very high. And I would say incrementally, it's higher than where we were last year and a lot higher than where we were when we go back 2 or 3 years ago. And the reason for that is, we now have really good confidence and we can see the trajectory of our existing product portfolio and that's growing quite nicely. And while Eliquis is not part of that growth portfolio, Eliquis continues to perform very well. And that gives us a lot of confidence that, that business is going to continue to grow. It's got good momentum coming into this year. So that's a big piece of the confidence that I've got. And then I would say, in spite of the setbacks that we had, look, we've got a great team in the development organization. Cristian has brought in new energy and focus. So that's really good. And if you look at the sheer number of

opportunities that we have, I highlighted the opportunity for 6 new medicine pivotal data this year alone. And if you just expand the aperture just a bit, you have the opportunity for upwards of 10 new medicines between now and the end of the decade with over 30 meaningful life cycle management opportunities. There's just a lot of shots on goal there and you don't have to wait very long to see how that's going to play out because you've got a very large swath of those pivotal studies that are going to start reading out in the middle of the year. So that gives me a lot of confidence. And I can't tell you how important it is to be financially in the place that we are, to be very disciplined in how we've operated, to have great cash flow. Our balance sheet looks good. We've drawn down our debt. And that really puts us in a strong position as a company and that gives us optionality.

Christopher Schott

Just, I guess, on that pipeline front, I mean, it is a big year coming in, '26. You laid out all those assets reporting out. What is, I guess, a good enough outcome look like for you? You've laid out some kind of growth objectives as you kind of exit the LOE cycle. Just elaborate a little bit more about what has to work here to support those growth objectives? And maybe where is your highest level of confidence of these assets reporting out?

Christopher Boerner

Well, the good news is that we've intentionally built this portfolio to be diversified, broad and replete with assets that individually have significant commercial potential. And I say that because in some ways, what that implies, to sort of reframe it is, we don't need everything to work. We have a lot of confidence in the programs that are going to be reading out starting this year. But this is a highly diversified set of assets that we feel good about each of the chances individually but we're not beholden to all of these programs working. So that's an important point right upfront. What I would say about the swath of assets that are coming forward is, if you look at the clinical and scientific rationale just behind the 6 new product potential data readouts that we have this year, milvexian, we've got very good Phase II data across both of those indications, SSP and AFib. We've seen competitor data for SSP, their Phase III. So that gives us a lot of confidence in the program and we think we have a better asset. We've got a lot of rationale behind Cobenfy and we can talk more about that. Admilparant, 2 very strong Phase II datas in terms of efficacy leading into the IPF and PPF program. So you kind of go down the list, you have a lot of confidence in these programs. And I think that, obviously, we need to see the science play out here but it gives me a lot of confidence these programs are going to be an important catalyst and second leg of the stool, if you will, for the growth of the company.

Christopher Schott

There's a couple of other bigger picture one. Maybe before we go into the individual assets, BD priorities, I mean you talked about financial flexibility. Where are you most focused at this point from a BD perspective?

Christopher Boerner

Well, business development remains a top priority for how we think about capital allocation. We're going to cast a broad net but that net really is focused on how can we improve the growth profile of the company and deliver on that objective I talked about earlier. What that means practically is, obviously, we're going to focus on areas we know well. We've got opportunities to build depth across each of our core therapeutic areas. While we will look at opportunities across Phase I, II and III to the extent that we can index on opportunities that give us near-term growth, that's going to be important. But we're going to continue to be very financially disciplined in how we go about that. So we're going to look for ways that we can say we're the rightful owners of these assets. It makes scientific sense for us to be a participant. And ultimately, we've got to be able to stand in front of investors and you and others and say, this is going to add value to the company. But if those criteria are met, we've got the financial capital to be part of the process.

Christopher Schott

So it sounds like BD may be pivoting a little bit later stage just given the financials of the company.

Christopher Boerner

I mean we're still building for the long term. So we see really attractive Phase II opportunities. For example, we're going to be -- and it makes sense for us to be the owners, we'll be in the mix.

Christopher Schott

And in terms of therapeutic areas, should we think about building on where the company currently has presence? Or could we be looking at more Karuna like deals where you're kind of adding adjacent vertical or a new vertical?

Christopher Boerner

I think our primary focus is going to be to continue to build depth in the core therapeutic areas that we're in and we think we've got opportunities to do that really across each of those TAs.

Christopher Schott

Okay. Great. Maybe just shifting to the pipeline. You mentioned R&D organization, you've made some changes. Can you just elaborate a little bit more in terms of what's changed here? And when can we start to see some of the outcome of those changes being implemented?

Christopher Boerner

Well, look, I think I would position everything that we've done in the context of R&D with what we've done in the broader organization. We have been fixated on ensuring that we're driving execution across the entire company. We have something internally we call our say-to-do ratio. When we say we're going to do something, investors should have confidence that we're going to deliver on it more often than not. And that's become a mantra for the company across the board. And obviously, when you see that you have issues that pop up, you got to move quickly to address them. We've done that in the context of

development. Cristian has brought in a new energy and perspective there. And I think, again, the focus is on making sure we're focusing on the right areas of science, that we're focused on areas that we know well and we can add value to, building depth in each of those TAs. Execution is super important. So we've taken a hard look across all of our clinical programs, making sure that we're doing everything we can to deliver those programs not only on time but with the highest probability of success. Those efforts are well underway and we're seeing the benefit of those already. And then finally, taking a hard look at the portfolio to ensure that everything that we're investing in not only has clear scientific rationale but can deliver commercial value. We started that effort back in 2024. And I think any time we talk internally about research and development, right next to that is portfolio prioritization, which we're going to continue to be focused on. And again, you saw us externalize some assets in 2025. And we're going to continue to sort of leverage that portfolio effort to understand where we need to be making more investments, where we need to be looking for business development. And obviously, when appropriate, when there are things we should stop investing in.

Christopher Schott

Great. Going into individual products, maybe first starting on milvexian. We got secondary stroke and AF coming this year. Just maybe to start with how meaningful of a product could this be for Bristol, maybe specifically on that AF indication if it were to hit. Can you just talk about what you need to see with the profile and what is the opportunity set for the drug in that situation?

Christopher Boerner

Well, I think if you look at the components of both of the milvexian programs, they have everything you look for in terms of becoming a very meaningful asset. First, there's unmet need across both SSP and AFib. And with AFib specifically, while products like Eliquis have been incredibly successful and the Factor Xas have made a significant difference in AFib, it's helpful to remember, 40% of patients either aren't treated with a Factor Xa or they're undertreated because of a concern of bleed. So there's clearly an opportunity to layer on top of standard of care for those patients. Second, you've got really strong clinical rationale for the development of milvexian in both of those indications. We've got Phase II data for SSP. We have a very attractive Phase II program that read out positively in total knee replacement, which I think gives us an indication of how it will perform in AFib. And remember, in spite of a competitor program failing fairly early, this program continued to enroll and enrolled quickly. We think we did the right study and there's been a lot of excitement amongst KOLs in enrolling those patients. So we feel good about the clinical rationale. And then while SSP is a smaller disease area, these are big commercial opportunities, about just shy of 1 million patients with risk of secondary stroke and 17 million patients with AFib. So that's really big, even if you just consider the 40% that are under or not treated with Factor Xa. So add those things up, this is going to be a potentially quite significant opportunity for us.

Christopher Schott

Maybe turning to Cobenfy. Can you just walk us through some of the recent trial amendments we saw with the ADEPT-2 study in Alzheimer's psychosis? Basically how much clarity does Bristol have at this point in

terms of the profile of Cobenfy that -- from -- in that study?

Christopher Boerner

Well, remember, when you talk about Cobenfy, it's important to recognize that we've got a broad development program. I highlighted a number of the programs that are going on. We've got Alzheimer's disease psychosis. I'll dive into that in a second. But we've got Alzheimer's agitation and cognition, bipolar and autism. So this is a broad program that we're developing. With respect to ADP, 3 studies this year. We actually have 4 studies on ADP. We've got 1, 2 and 4 reading out this year. We actually have a fifth ADEPT study that's a bridging study that reads out a bit later. We believe we're going to need 2 of those studies in order to file and that's typical in the space. With respect to ADEPT-2, look, I think what we've been very careful about with this program, is making sure that we remain blinded to the data, that we maintain the integrity of the study. We worked very closely with FDA and the DMC on everything that we've done. And what do we know? Well, first, we know the relapse rates are consistent with what our expectations were. We know the fact that the DMC, when they looked at the data unblinded, they recommended that we continue to enroll and enroll up to the original patient numbers. And so that's what we're doing. And we anticipate that the study is going to read out roughly in the same time frame that ADEPT-1 and ADEPT-4 would read out. So it's going to be an important year for that program. And if you just step back one more step and look at the rationale for Cobenfy in this space, that we feel -- still feel really confident about. We've got internal data, whether it's that relapse rate that I referred to earlier, the lead-in to ADEPT-1, we've seen that data. And then, of course, we've got external data going back to the early Lilly data that provides proof of concept here. So add it all up, there's confidence but we've got to see the studies read out.

Christopher Schott

Great. Just to maybe also help frame out the opportunity for ADP as -- to the extent we get 2 ADP studies to work, like just compare and contrast how that might look relative to the schizophrenia indication you've been launching?

Christopher Boerner

Well, this is obviously a big patient population. You have 6 million patients more or less with, diagnosed with Alzheimer's disease. We believe roughly 40% of those patients have psychosis symptoms defined by hallucinations or delusions. So it's a big patient population. There's significant unmet need. Most of those patients, if they're being treated, they're being treated with D2s. Those D2s don't work particularly well and have the added problem that there's a black box warning for mortality in the elderly, clearly not something you want to see in Alzheimer's. And so if the profile that we believe exists here pans out, this is a significant opportunity and there's going to be excitement about it. And of course, then the focus will be on continuing to go and identify those patients and then build breadth and depth. And so I think if the study is positive, there's significant opportunity here.

Christopher Schott

And would this be only, think about a gradual ramp with it? Or could this be a little...

Christopher Boerner

I think so. I mean I think this is generally a space where physicians are relatively conservative and I think with good data, it's -- there's a lot of blocking and tackling as we're seeing in the schizophrenia space in terms of getting physicians to try a product. But with the right profile, once they try it, they build depth pretty quickly.

Christopher Schott

And speaking of schizophrenia, talk a little bit about progress you've made there and kind of key priorities for that indication.

Christopher Boerner

We feel very good about where we are with schizophrenia. And as I think we've discussed previously, this has all of the core components of a very big product. It's got very good clinical data. Once physicians try the product and see the benefit for themselves and more importantly, hear what comes back from patients, again, it's one where they start to build depth pretty quickly. The trick is, it's a conservative physician group. And so you've got to make sure that you're able to get them to do that first try. But there's no impediment to the commercial uptake of this product. So access is very good. The profile is panning out exactly as we thought. We're going to have data coming into the market this year that will help on dosing and switch. So we think that data is going to be important to continue to accelerate the launch. We doubled the size of our commercial sales force to increase the shots on goal that we have in terms of talking to physicians about the efficacy profile. So you add it all up, I think you're going to continue to see a nice steady uptake with this product. And again, what you don't want to see are concerns coming forward that the profile isn't what you expected. And that's not the case here. When physicians and patients see the benefit of this product, they want to not only use the product but patients want to stay on the product and that's super important.

Christopher Schott

And sounds like steady is the way to think of this versus...

Christopher Boerner

Steady is the way to think about it.

Christopher Schott

Perfect. Pivoting over to oncology. The CELMoDs, seems like this one could be a bit of a sleeper, less focused on by the Street and you've got a lot of confidence. Can you just help frame out the opportunity for those assets as we start to think towards the data this year?

Christopher Boerner

Yes. I think you've characterized it well. Look, I was in the field talking to physicians about the opportunity that we have with iber and mezi. There's a lot of excitement about it. Remember, multiple myeloma, it's obviously a crowded space but patients cycle through lines of therapy and products relatively quickly. And many, if not most, of these patients continue to be treated in the community setting where the opportunity to have an oral therapy that combines well with standard of care is a really attractive proposition. And what's nice about what we're seeing with the evolution of this program is, leading into the Phase IIIs, we saw very good Phase II data. We now have the MRD negativity data for iber, which looks very, very good. And that portends success and an attractive profile when we see the PFS data, which will happen this year. And then we've got the first of the Phase IIIs with mezigdomide this year. And this is a broad program. They're combinable assets. So again, it's one where when we see the data continue to evolve this year for both of those programs, I think if they play out the way we anticipate they will, these are going to be exciting opportunities. And I think we have a real opportunity to play in multiple spaces within multiple myeloma and help a lot of patients.

Christopher Schott

When I think about the CAR-Ts and bispecifics in the setting coming in, does that complicate how these drugs will find the kind of role in the market just given the standard of care is changing so quickly?

Christopher Boerner

Well, I think the standard of care is changing very quickly. But I think that it's a complicated space. But as I referenced before, what we like about CELMoDs generally is, they're combinable. So you can combine them not only with standard of care monoclonal antibodies, for example but you can combine them with cell therapy. You can combine them with T-cell engineered antibodies. And so while it's a complicated space, in some ways, that plays nicely to the CELMoD program and in particular, what we're seeing with iber and mezi. So look, I was pleasantly surprised to see the level of enthusiasm to see this data precisely because I think it provides another bullet in the armamentarium that physicians have to treat these patients. It's combinable. The data looks pretty good that we're seeing. And now, of course, we've got to continue to see the data play out. Good news is, here again, you don't have to wait very long. You're going to start to see more data on both of these programs this year into next year and the following year.

Christopher Schott

Okay. And maybe one last one on the oncology front. The PD-1 VEGF, very exciting space. How do you think about Bristol differentiating within the category? It seems like there's a number of other players. It seems like every day another one kind of coming into the space, I know you're one of the earlier ones. But how do you think about kind of uniqueness of the asset and the clinical program you're running?

Christopher Boerner

Well, we've obviously seen not only data with the asset that we're working on with BioNTech but also you continue to see an evolution of the data in the space. And I will tell you, we continue to be attracted to the data that we're seeing. This is data that has really good activity in very difficult-to-treat patients. So our

conviction that this could be a new platform therapy, I-O therapy continues to be very strong. So differentiator #1. We like the data that we're seeing in general and we like the data that we're seeing with this asset specifically. Differentiator #2, this is a space we obviously know very well. I've gotten to know the team at BioNTech quite a bit over the last year plus. And what I would say is that you combine the scientific expertise that they have, not only with this product but also with their broader suite of oncology assets, along with the incredibly deep experience that we have in the I-O space, you put those 2 teams together and have a partnership that we've built over the last year, which is very, very strong. I think that's differentiator #2. Differentiator #3 is, we think we are in a prime position to be first or second in each of the disease areas that we're targeting within oncology. And what do we know from our existing position in I-O is that being third, fourth or fifth is pretty challenging. And that's challenging today, never mind what it's going to look like when these products launch. So we really believe that being first or second is super important to driving value. And I think with the expertise that we have across the 2 companies, we're in a good position to do that. And if the science evolves, we're in a great spot with this partnership and this could be a meaningful opportunity.

Christopher Schott

Great. Last couple of minutes here. Maybe talk about 2026. I know you haven't formally given guidance but just kind of pushes and pulls we should all be considering as we think about the year ahead.

Christopher Boerner

Yes. We'll obviously provide more details about 2026 in a few weeks. But what I would say just generally, objective remains the same. Our focus is on continuing to build a strong company, a strong company that's very financially disciplined and a company that continues to be focused on how do we put all of the pieces into a place to be a company that can deliver sustained long-term growth exiting the decade into the next. How do we do that? Commercial execution and delivering on our financial commitments, priority #1. We've got good momentum with that growth portfolio coming into this year. So I feel good about that. But that -- executing on that is super important. Second, obviously, with the wealth of assets we have coming in the pipeline, we got to deliver on those. So there's a lot of focus that we have in the company on good, solid blocking and tackling execution on the development side, more generally across R&D. So that's the second thing. Third, we've got to continue to be smart about how we allocate our capital. So we've talked about business development. That continues to be a priority for us and being smart about the opportunities that we choose to invest in. I think that's going to be important and that's a priority. And last but not least, I'm very proud of the changes that we've made in the organization with respect to being much more focused on execution, being much more disciplined in how we operate, rewiring to be more efficient. That includes continuing to invest in technology. And given the opportunity we have, given this moment in time at the company, we're going to continue to push hard on that and that's going to be a focus area as well.

Christopher Schott

Great. One last one for me. MFN tariffs, I know Bristol recently did a deal with the administration. Just your perspective on what that means for the company and maybe for the industry more broadly, that

agreement?

Christopher Boerner

Well, we signed the deal. The nice thing about our portfolio, remember, because we have relatively limited Medicaid exposure, so you guys can do the math in terms of how you think about the impact of that. What I'm really excited about is that we now have the ability to start focusing -- we talked about this earlier, on value creation and hopefully begin thinking about partnering not only with this administration but thinking about this more globally on -- in the U.S., how do we continue to simplify the ecosystem in the United States? Because if ultimately, you want to increase access, increase affordability, you've got to take costs out of the system. And the way you do that is make this a simpler environment within which to operate. And there, I think the administration has been quite supportive in terms of finding ways to sort of circumvent third parties and middlemen. So we're going to continue to focus on that. And then I would say a second area where the administration has been pretty helpful is in ensuring that countries outside of the United States pay their proper share for the innovation largely that's taking place here. So we'll continue to engage there. But I feel good that we've gotten beyond that deal and now we can start focusing more on fundamentals. And the great thing about where we are as a company, we have a lot of fundamentals to focus on. We've got good commercial execution and a lot coming in the pipeline. And I'd leave you with -- if there's one thing that we're going to be focused on, is ensuring that we deliver that swath of new opportunities because there's a lot coming.

Christopher Schott

Great. Well, appreciate the time and exciting year ahead. So thanks for the comments.

Christopher Boerner

Thanks. Happy to be here.